KNEE HEALTH CHECK

Your Knee Care Guide

Understand your score. Protect your knees. Stay active, at every age.

A complete guide for all score bands — exercises, daily habits, pain relief, knee supports and when to see a doctor.

START HERE

How to use this guide

This guide covers all three Knee Health Score bands. Find your score below, read the section for your band, then use the common sections that follow — they apply to everyone.

Your score	What it means	Read these sections
0–30%	Your knees are in good shape	Section 2 (green band), then 3, 4, 5, 8, 9
31–60%	Early warning signs	Section 2 (amber band), then 3 to 9
61–100%	Strong signs of knee arthritis	Section 2 (red band), then 3 to 10

Please see a doctor first if any of these apply

- The knee is suddenly swollen, red or hot
- You have a fever along with knee pain
- You cannot put weight on the leg
- The knee locks, or gives way after a fall or twist

These need a doctor's attention before you start any exercise in this guide.

What is inside

Section	What it covers
1. Understanding your knees	How the knee joint works and what arthritis means
2. What your score means	Your band, and the first steps for it
3. Your exercise programme	Seven exercises, with repetitions for each band
4. Everyday habits	Floor sitting, stairs, toilets, footwear, travel
5. Managing pain day to day	Heat and cold, pacing, medicines, weight
6. Knee supports explained	Knee cap, hinged brace and off-loading brace
7. Myths and facts	Common beliefs about knee pain in India
8. When to see a doctor	Warning signs and urgent signs
9. Questions to ask your doctor	A checklist to take to your appointment
10. Your first four weeks	A simple week-by-week plan

SECTION 1

Understanding your knees

The knee is where the thigh bone meets the shin bone. The ends of both bones are covered with cartilage, a smooth layer that lets the joint glide and cushions it as you walk. Strong thigh and hip muscles share the load and act like shock absorbers.

In osteoarthritis, the cartilage gradually becomes thin and rough. The joint space narrows, the bone underneath reacts, and the knee starts to feel painful, stiff or noisy. It usually develops slowly over years, and it often affects one side of the knee more than the other — most commonly the inner side, which carries more of your body weight.

Why it becomes common after 45

- **Age:** cartilage naturally becomes thinner with the years.
- **Body weight:** extra weight multiplies the load passing through the knee with every step.
- **Past injury or surgery:** a torn ligament or meniscus, or an old fracture, raises the risk.
- **Family history:** arthritis often runs in families.
- **Years of deep bending:** long hours of squatting, kneeling or floor sitting add up.
- **Weak thigh muscles:** when the muscles weaken, more force passes through the joint itself.
- **For women:** knee arthritis becomes more common after menopause.

The most useful thing to know

Knee arthritis cannot be reversed, but pain and stiffness can very often be reduced. Strong muscles, a healthy weight, sensible daily habits and the right knee support help most people stay active for years. Surgery is not the first step, and for many people it is never needed.

Words your doctor may use

Word	What it means in simple language
Osteoarthritis	Wear and thinning of the cartilage in the joint
Joint space narrowing	The gap between the bones on X-ray has reduced
Medial compartment	The inner side of the knee, the part most often affected
Crepitus	The grinding or cracking sound you hear when the knee moves
Varus	Bow-legged shape, where more load passes through the inner side
Conservative treatment	Treatment without surgery: exercise, weight care, braces, medicines

SECTION 2

What your score means

Your score adds up your age, symptoms, daily difficulties and history. It is an indicator, not a diagnosis — only a doctor can confirm arthritis.

0–30%

Your knees are doing well

Your answers do not show many common signs of knee arthritis. This is the best time to protect what you have.

Your first steps

- Do the exercise routine in Section 3 most days of the week.
- Walk 20 to 30 minutes a day on flat, even ground.
- Keep your weight in a healthy range and wear cushioned footwear.
- Take the check again in six months, or sooner if pain starts.

31–60%

Early warning signs

Your answers show some signs often seen at the start of knee arthritis. At this stage, small changes make a big difference, and most people improve without surgery.

Your first steps

- Start the exercises in Section 3 gently, five repetitions at first.
- Make the daily changes in Section 4, especially floor sitting and stairs.
- Get your knee examined by an orthopaedic doctor in the next few weeks.
- Ask about a knee support for long walks and stairs (Section 6).

61–100%

Strong signs of knee arthritis

Your answers match many common signs of knee arthritis. This needs a proper check-up, but it does not mean surgery — most people at this stage have several treatment options.

Your first steps

- See an orthopaedic doctor and ask for a standing X-ray.
- Take the checklist in Section 9 with you, and begin the gentle exercises in Section 3 once your doctor agrees.
- Ask whether an off-loading knee brace suits your type of arthritis (Section 6).

SECTION 3

Your exercise programme

Exercise is the single most effective thing most people can do for knee arthritis pain. The aim is to strengthen the muscles around the knee and hip so they take load off the joint. Do these once a day. Each exercise lists repetitions for each score band.

Five rules before you start

- Move slowly and breathe normally. Never hold your breath.
- Mild muscle tiredness is normal. Sharp pain in the joint is not — stop that exercise.
- Some soreness for a day is fine. If pain lasts more than 24 hours, do fewer repetitions next time.
- Do both legs, even if only one knee hurts.
- If you have had knee surgery, a heart condition or severe pain, ask your doctor first.

1

Thigh tightening (quadriceps sets)

Sit or lie with your leg straight in front of you. Tighten the muscle on the front of your thigh so the back of your knee presses gently down towards the floor or bed. Hold for 5 seconds, then relax fully. This is the safest starting exercise and can be done even on painful days.

0–30%: 10 each leg | 31–60%: 5, build up to 10 | 61–100%: 5 each leg

2 **Seated knee straightening**

Sit on a firm chair with your feet flat. Slowly straighten one knee until the leg is out in front of you. Hold for 5 seconds, feeling the thigh muscle work, then lower slowly. Do not swing the leg.

0–30%: 10 each leg | 31–60%: 5, build up to 10 | 61–100%: 5 each leg

3 **Heel slides**

Lie on your back with both legs straight. Slowly slide one heel towards you, bending the knee as far as is comfortable, then slide it back. This keeps the knee's movement free.

0–30%: 10 each leg | 31–60%: 5, build up to 10 | 61–100%: 5 each leg

4 **Straight leg raise**

Lie on your back with one knee bent and the foot flat. Keep the other leg straight, tighten the thigh, and lift that leg to the height of the bent knee. Hold for 3 seconds and lower slowly. Keep the knee straight throughout.

0–30%: 10 each leg | 31–60%: 5, build up to 10 | 61–100%: Only if comfortable

5 **Side leg raise (hip strength)**

Lie on your side, or stand holding a chair. Lift the upper leg out to the side, keeping it straight and your toes pointing forward. Hold for 2 seconds and lower slowly. Strong hip muscles reduce the load on the knee.

0–30%: 10 each leg | 31–60%: 5, build up to 10 | 61–100%: 5 each leg

6 **Sit to stand**

Sit on a firm chair with your feet flat and slightly back. Stand up slowly without using your hands if you can, then sit down slowly with control. Keep your knees in line with your toes. Use the armrests if you need to.

0–30%: 10 | 31–60%: 5, build up to 10 | 61–100%: Use armrests, 5 times

7 **Heel raises**

Stand behind a chair and hold the back for balance. Rise slowly onto your toes, hold for 2 seconds, then lower your heels with control. This strengthens the calf, which helps you walk and climb stairs.

0–30%: 10 to 15 | 31–60%: 10 | 61–100%: 5 to 10

Walking

Walking is exercise too, when it is done sensibly.

- Walk on flat, even ground. Avoid uneven roads, steep slopes and hard downhill walking.

- Two shorter walks are kinder to a sore knee than one long walk.
- If pain rises during a walk, stop, rest and do less next time rather than pushing through.
- Swimming and cycling on a stationary cycle are gentle options if walking is painful.

SECTION 4

Everyday habits that protect your knees

Most knee load in daily life comes from a few repeated activities. Changing how you do them gives the joint a rest many times a day.

Sitting and getting up

- Use a chair for meals, prayer, TV and guests when you can. A chair with armrests and a firm seat is easiest.
- If you sit on the floor, use a cushion to raise your hips, keep one leg less bent, and get up with support.
- Avoid staying in one position for more than 30 minutes. Stand, take a few steps, and sit again.
- Choose a seat height where your hips are slightly higher than your knees.

Toilet and bathroom

- A western toilet puts far less strain on the knees than an Indian-style toilet. A raised seat or commode chair over the existing one works well.
- Fix a grab bar on the wall beside the toilet and inside the bathroom.
- Use a non-slip mat. A slip is the most common cause of a sudden knee injury at home.
- A plastic stool for bathing avoids deep squatting.

Stairs and temple steps

- Always hold the railing.
- Going up, lead with your stronger leg. Coming down, lead with the painful leg.
- Take one step at a time when the knee is sore. There is no benefit in hurrying.
- At temples with long flights of steps, plan rest points and go early when it is cooler and less crowded.

Footwear, home and travel

- Wear footwear with a cushioned sole and good grip, including indoors if your floors are hard.
- Avoid worn-out slippers with flattened soles, and avoid heels.
- Keep everyday items between waist and shoulder height so you bend and stretch less.

- On long journeys, straighten and bend your knees every half hour, and walk a little whenever you stop.
- Carry a light folding stool if you attend long functions or pilgrimages.

If you use a walking stick

Hold the stick in the hand **opposite** your painful knee, and move the stick forward at the same time as the painful leg. The stick should reach your wrist height when you stand straight. Used this way, a stick takes real load off the knee — it is a tool, not a sign of weakness.

SECTION 5

Managing pain day to day

Heat and cold

Use	When	How
Warm pack	Stiff knee, especially in the morning or before exercise	15 minutes, wrapped in a cloth. Do not use heat on a swollen knee.
Cold pack	Sore or swollen knee after activity	10 to 15 minutes, wrapped in a towel. Never place ice directly on the skin.

Take extra care

If you have diabetes, poor circulation or reduced feeling in your legs, ask your doctor before using heat or cold packs. You may not feel a burn or a cold injury until it has already happened.

Pacing your day

- Split heavy tasks. Do the kitchen work in two sittings instead of standing for an hour.
- Alternate a demanding task with a light one so the knee gets short rests.
- Plan the walking-heavy part of the day for when your knee feels best, usually late morning.
- Rest is useful in short spells. Long bed rest makes muscles weaker and pain worse.

Medicines

- Take pain medicines, gels or injections only as prescribed by your doctor.
- Long-term use of painkillers without medical advice can affect your stomach, kidneys and blood pressure.
- Tell your doctor about every other medicine you take, including for diabetes, blood pressure or the heart.
- Pain balms and gels are useful for short-term relief and are gentler on the stomach than tablets.

Weight and food

- Even a small, steady weight loss reduces the load your knees carry with every step.
- Aim for gradual change through everyday food: smaller portions of rice, more vegetables and protein such as dal, curd, eggs, fish or paneer.
- Avoid crash diets. Losing muscle along with weight leaves the knee less supported.
- Ask your doctor before starting calcium, vitamin D or joint supplements. No supplement rebuilds worn cartilage, whatever the advertisement claims.

SECTION 6

Knee supports explained

“Knee cap” is used loosely in India for very different products. They do different jobs, and the right one depends on your type of pain.

Type of support	What it does	Who it usually suits
Knee cap or sleeve (elastic, no hinges)	Gives warmth, compression and a feeling of support. Does not change the load inside the joint.	Mild pain, swelling after activity, extra confidence on walks
Hinged knee brace (metal or plastic supports at the sides)	Adds side-to-side stability and controls how far the knee bends.	Ligament problems, after an injury or surgery, a knee that feels unsteady
Off-loading brace (also called an unloader brace)	Gently shifts pressure away from the worn side of the joint, so the painful side carries less load.	Arthritis affecting mainly one side of the knee, often the inner side, with bow-legged shape

How an off-loading brace works

When arthritis affects mainly the inner side of the knee, that side carries more load with every step, which is why pain is often felt there. An off-loading brace applies gentle pressure at three points along the leg to open the worn side slightly, so it carries less load. Many people find they can walk further with less pain while wearing it. Your doctor decides whether it suits you, based on your examination and X-ray.

Getting the fit right

- Measurements matter. Thigh and calf measurements decide the size, not your shoe or waist size.
- Wear it over a thin cotton sleeve or leggings to protect the skin.
- Start with one to two hours a day and build up as your skin and muscles adjust.
- It should feel firm but never painful. Redness that does not fade in 20 minutes means it needs adjusting.
- Wear it for the activities that hurt most: walking, stairs, market visits or standing at work.

A brace does not weaken your muscles

A common worry is that wearing a support makes the knee lazy. Used as advised, alongside the exercises in Section 3, a brace usually lets you walk and move more — which makes the muscles stronger, not weaker.

Find the best knee brace for your knee pain

Speak to our knee care team for free. We will understand your pain, explain your options and help you choose the right support.

BOOK MY FREE CALL

Or WhatsApp **CALL** to **+91 91500 64620**
and we will call you back.
Free · No obligation

SECTION 7

Myths and facts

What people often say	What is actually true
“Walking will wear out my knees faster.”	Sensible walking on flat ground helps. It keeps the joint moving and the muscles strong. Complete rest makes knees stiffer and weaker.
“The cracking sound means damage.”	A painless sound is usually harmless. It matters when it comes with pain, swelling or the knee giving way.
“Knee pain means I will need a knee replacement.”	Most people with knee arthritis are managed without surgery. Replacement is considered when pain is severe and other treatments have not worked.
“Calcium tablets will fix my arthritis.”	Calcium and vitamin D support bone health. They do not rebuild worn cartilage or treat arthritis.
“Oil massage cures arthritis.”	Massage may feel soothing and help you relax, but it does not change the joint itself.
“I should wait until the pain is unbearable to see a doctor.”	Early treatment gives more options. The earlier you start exercise and weight care, the better the result.

SECTION 8

When to see a doctor

Book an appointment if

- Knee pain on most days for more than a month
- Pain that disturbs your sleep
- Swelling in the knee, or the knee feels unsteady
- Pain that stops you from doing daily activities
- Morning stiffness that lasts more than half an hour
- Pain in both knees, or in other joints as well

See a doctor urgently if

- The knee is suddenly swollen, red or hot
- You have a fever along with knee pain
- You cannot put weight on the leg
- The knee locks, or gives way after a fall or twist
- There is pain, numbness or tingling travelling down the leg

What usually happens at the appointment

- The doctor asks about your pain, how long you have had it, and what makes it worse.
- The knee is examined for swelling, tenderness, movement and stability.
- A standing X-ray of both knees is often advised. Standing matters, because it shows the joint under load.
- An MRI is not needed for most cases of knee arthritis.
- Together you decide a plan: exercises, weight care, medicines, a knee support, or a specialist referral.

SECTION 9

Questions to ask your doctor

Print this page or keep it open on your phone during the appointment. Ticking off answers helps you remember what was said.

- [ ] What stage is the arthritis in my knee?
- [ ] Which side of my knee is most affected?
- [ ] Which exercises are safe for me, and which should I avoid?
- [ ] Would a knee brace help me? Which type?
- [ ] How much weight should I aim to lose, and over what period?
- [ ] Which pain medicine is safe for me, considering my other medicines?
- [ ] Do I need physiotherapy? How many sessions?
- [ ] Do I need knee replacement now, or can I try other treatments first?
- [ ] What should I do if the pain suddenly becomes worse?
- [ ] When should I come back for a review?

Write down the answers here

SECTION 10

Your first four weeks

A simple way to begin. Adjust it to your own pace, and to your doctor's advice.

Week	Exercises	Daily habits	Other
Week 1	Exercises 1 to 3, once a day	Set up a chair for meals and prayer. Check your footwear.	Book a doctor's appointment if your score is 31% or above.
Week 2	Add exercises 4 and 5. Same repetitions.	Start the stairs routine: railing, stronger leg up, painful leg down.	Note which activities hurt most — useful for your doctor.
Week 3	Add exercises 6 and 7. Build repetitions towards 10.	Add a short daily walk on flat ground, or extend the one you do.	Ask your doctor about a knee support if pain limits walking.
Week 4	Full routine once a day.	Add the bathroom and travel changes from Section 4.	Take the Knee Health Check again and compare your score.

How to know it is working

- You can walk a little further before the pain starts.
- Stairs and getting up from a chair feel easier.
- You need fewer pain tablets in a week.
- Morning stiffness eases faster than it used to.

Progress is usually gradual and shows over weeks, not days. If pain is getting worse instead, stop and speak to your doctor.

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